

Some women report a headache after an epidural, which can develop more than 24 hours

after the delivery and tends to be at the front of the head. It is made worse by sitting up and moving around and is much improved by lying down. This occurs in around 1 in 100 women and is caused by the epidural needle moving too far forward and cutting the dura sheath, the membrane maintaining the fluid around the spinal cord and brain. This small hole results in a loss of fluid from the sheath, which causes a headache. The risk is hugely reduced by remaining still during the placement of the epidural. In around 70 percent of women, the hole heals on its own. You will be advised to drink plenty of fluids and to take simple painkillers, such as acetaminophen and ibuprofen and you will be reviewed at regular intervals by an anesthesiologist. If the headache persists, a procedure called a “blood patch” will be done. This is done in the sterile environment of an operating room by two anesthesiologists. One places an epidural needle in your back, while the other takes around 20 ml of blood from a vein in your arm. The blood is then passed down the needle into the epidural space. This forms a clot that seals the hole and prevents further leakage of fluid from around your spine, therefore relieving the headache.

BIRTH STORY: EPIDURAL PAIN RELIEF

Alice was having her first baby. Her pregnancy had been uncomplicated and she had written a birth plan outlining her desire for a natural childbirth by keeping active and using TENS and then warm water to deal with contractions. She wished to avoid an epidural if possible.

Alice's birth story: My husband and I arrived at the delivery suite in early labor. I started to use a TENS machine for pain relief. However, as my labor progressed, I became very distressed since I hadn't anticipated that the contractions would be so painful. When I was around 3 cm dilated, I decided to remove the TENS machine and get into the bath. My husband gave me a back massage and provided emotional support. However, I think he struggled to understand my discomfort and he needed support from the nurse. After 15 minutes, I decided to get out of the bath since it was providing little pain relief. I used a birth ball to stay active and my husband gave me more massage and acupressure. I coped well for the next hour, but then became increasingly exhausted and upset. When I was examined, I was only 5 cm dilated. We both felt despondent because we had hoped I was further along.

My nurse then suggested that I talk to the anesthesiologist about my options for further pain relief, and as a result of my conversation with the anesthesiologist, I decided to have an epidural. I told the anesthesiologist that I'd had an epidural a few years ago for knee surgery and how it had provided excellent pain relief, but that I had itched for hours afterward. The anesthesiologist surmised that the itch was caused by one of the painkilling medications (fentanyl) in the epidural top-off and agreed that this medicine wouldn't be used.

The anesthesiologist agreed to do a low dose combined spinal epidural that gave absolute pain relief within five minutes. My legs were a little heavy at first, but they felt fine within an hour. I felt that we were both able to take time out after the epidural and that I was able to refocus on my labor. I felt pleased that I'd managed a large part of my labor without pain relief, and was happy with the decision to have an epidural when I did. I had an unassisted delivery later that evening.

The anesthesiologist comments: Alice kept an open mind regarding pain relief and understood that different methods of pain relief could be used at different times during labor. After the epidural, she no longer felt that her labor was an endurance test and was able to focus again on her labor and on delivering a healthy baby.